

SUSPECT ADVERSE REACTION REPORT

CIOMS FORM I: CASE 4

I. REACTION INFORMATION			
1. PATIENT INITIALS (first, last) Not Reported	1a. COUNTRY India	2. DATE OF BIRTH Not Reported	2a. AGE 11 months old
3. SEX Male	4-6. REACTION ONSET Day 1 (Coincides with the start of the incorrect formulation administration)		
8-12. CHECK ALL APPROPRIATE TO ADVERSE REACTION ☐ PATIENT DIED ☐ INVOLVED OR PROLONGED INPATIENT HOSPITALISATION ☐ INVOLVED PERSISTENCE OR SIGNIFICANT DISABILITY OR INCAPACITY ☐ LIFE THREATENING			
7 + 13. DESCRIBE REACTION(S) (including relevant tests/lab data) Reaction Description: The patient experienced an accidental drug overdose due to a dispensing and medication error. The physician prescribed Syrup Dolopar 125, but the pharmacist dispensed double the intended strength—Syrup Dolopar 250—due to an illegible prescription. The patient was administered 5 ml of the 250 mg / 5 ml formulation every 6 hours (4 times a day) orally. This resulted in an actual dose of 250 mg per administration (31.25 mg/kg per dose) and a cumulative daily dose of 125 mg/kg/day, which significantly exceeded the recommended maximum pediatric dose of 90 mg/kg/day. Lab Values: None reported. Outcome/Intervention: The error was detected timely by the patient's father on Day 2 after the child had received a total of 6 doses. The suspect drug was immediately discontinued upon detection. Because the error was intercepted before the cumulative dose reached the acute toxic threshold (>140 mg/kg/day) or persisted for multiple days, the patient did not develop clinical signs of hepatotoxicity or clinical toxicity and fully recovered.			
II. SUSPECT DRUG(S) INFORMATION			
14. SUSPECT DRUG(S) (include generic name)	Acetaminophen / Paracetamol (Dispensed as Syrup Dolopar 250)		
15. DAILY DOSE(S)	250 mg per dose, administered 4 times a day (Cumulative: 1000 mg/day or 125 mg/kg/day)		
16. ROUTE(S) OF ADMINISTRATION	Oral		
17. INDICATION(S) FOR USE	High grade fever and running nose (Common cold)		
18. THERAPY DATES (from/to)	Start: Day 1 Stop: Day 2 (Discontinued after 6 doses)		
19. THERAPY DURATION	2 days (Discontinued after 6 doses)		
20. DID REACTION ABATE AFTER STOPPING DRUG?	☐ YES ☐ NO ☒ NA Positive Dechallenge: No clinical symptoms of toxicity had manifested at the time of discontinuation / timely interception		

21. DID REACTION REAPPEAR AFTER REINTRODUCTION?	☐ YES ☐ NO ☒ NA Rechallenge not performed / Not ethically feasible
III. CONCOMITANT DRUG(S) AND HISTORY	
22. CONCOMITANT DRUG(S) AND DATES OF ADMINISTRATION (exclude those used to treat reaction)	None reported
23. OTHER RELEVANT HISTORY (e.g. diagnostics, allergies, pregnancy with last month of period, etc.)	Patient presented with a history of common cold, high-grade fever, and running nose. Patient's weight was recorded at 8 kg. No other medical conditions, allergies, or baseline comorbidities were reported.
IV. MANUFACTURER INFORMATION	
24a. NAME AND ADDRESS OF MANUFACTURER	Unknown Literature report
24b. MFR CONTROL NO.	Unknown Literature report
24c. DATE RECEIVED BY MANUFACTURER	Unknown Literature report
24d. REPORT SOURCE ☐ STUDY ☒ LITERATURE ☒ HEALTH PROFESSIONAL ☒ CONSUMER (Reporter Type: Father [Consumer] and Pediatrician [Healthcare Professional])	DATE OF THIS REPORT 15-Jul-2026 25a. REPORT TYPE ☒ INITIAL ☐ FOLLOWUP

Literature Citation Details:

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